

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 65 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: November 9, 2010 | Provider: Eleanor Park, MD — Internal Medicine | Doc ID: RFH-2010-1109-FU

Follow-up Office Visit

Review of Labs — New Diagnoses: T2DM & Dyslipidemia

Subjective

Returns to review fasting labs. Home BP averaging 138/86 on lisinopril 10 mg. Nocturia improved. Reduced sugary drinks; walking 15 min most days. Tolerating lisinopril without cough or dizziness.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
142/85	78	16	36.6 C	98% RA	95.1 kg	31.0

Assessment & Discussion

1. Type 2 diabetes mellitus, newly diagnosed (HbA1c 7.8%). Counseled on disease, diet, glucose monitoring, foot care.
2. Mixed dyslipidemia, high cardiovascular risk (smoker, diabetic, hypertensive). 10-yr ASCVD risk elevated — high-intensity statin indicated.
3. Hypertension — improved but above goal (<130/80 for diabetic).
4. Tobacco use — re-counseled.

Plan

- Start metformin 500 mg PO twice daily with meals; titrate as tolerated.
- Start atorvastatin 40 mg PO nightly.
- Increase lisinopril to 20 mg PO daily.
- Referral placed to Endocrinology (routine) and to Ophthalmology for baseline diabetic retinal exam.
- Home glucose meter prescribed; check fasting + 2-hr post-dinner.
- Pneumococcal and annual influenza vaccination administered today.
- Recheck HbA1c, CMP, lipids in 3 months. Repeat BMP in 2 weeks to monitor renal function/potassium after lisinopril increase.

Updated Medication List

Medication	Dose	Route	Frequency	Indication
Metformin	500 mg	PO	Twice daily	Type 2 diabetes
Atorvastatin	40 mg	PO	Nightly	Dyslipidemia / ASCVD risk
Lisinopril	20 mg	PO	Once daily	Hypertension
Aspirin	81 mg	PO	Once daily	Primary prevention